INFORMATION TO KEEP WITH YOU

# My complete treatment handbook

## Pemetrexed Monotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

**Displayed calendar**
**21 days**
See the active treatment phase

**Treatment stages**
**1**
Explained in the treatment reference

**Regimen source**
**NCCP 00222**
Version 5

Pemetrexed alone by drip on Day 1, repeated every 21 days. There is no preset number of cycles: your team reviews benefit and side effects.

## Treatment stages

01 **Pemetrexed alone: treatment, recovery and review**
Day 1 infusion every three weeks

Your team confirms the active stage and dates. The full treatment guide explains each step.

## Inside your handbook


| | | PAGE |
|---|---|---|
| 01 | Understanding your treatment | 2 |
| 02 | Your schedule, tests and appointments | 5 |
| 03 | Symptoms and supportive care | 10 |
| 04 | Your record and contact card | 19 |
| 05 | Sources and further information | 22 |


**My name** _____

**Hospital / oncology team** _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

# Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

## When to get help

**Emergency: call 112 / 999**

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

**Contact your oncology service immediately**

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call your oncology 24-hour number immediately for a temperature of 37.5°C or above, 36°C or below, shivering or sudden illness. Call urgently for new breathlessness, chest pain, bleeding, severe or persistent diarrhoea, vomiting despite medicines, inability to drink, very little urine or a spreading or blistering rash. Do not wait for the next visit; serious infection can occur without a high temperature.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

## Your chemotherapy medicine

Pemetrexed is chemotherapy used to control selected non-squamous non-small-cell lung cancer that is locally advanced or has spread. Maintenance aims to keep cancer controlled after platinum chemotherapy has finished without the cancer progressing. Second-line treatment is a separate treatment decision after previous chemotherapy. Ask which aim and indication apply to you.

ANTIFOLATE CHEMOTHERAPY

### Pemetrexed

Alimta



**Interrupts deoxyribonucleic acid (the cell's genetic material) (DNA) building-block production**

Pemetrexed blocks several enzymes that use folate to make the building blocks of DNA and RNA. This can slow or stop cancer-cell growth. Healthy cells can also be affected. Prescribed folic acid and vitamin B12 reduce important toxicities and are part of treatment.

Folate-dependent enzymes blocked → DNA building blocks reduced → Cell growth disrupted

**What to watch for**

Take the prescribed folic acid and attend for B12. Report mouth sores, diarrhoea, a rash, breathlessness or reduced urine; check before taking ibuprofen or other non-prescription medicines.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

## Understanding your treatment

Confirm the treatment aim and the medicines on your prescription.

### One medicine, its own plan

This guide covers pemetrexed on its own for maintenance or second-line treatment of selected non-squamous non-small-cell lung cancer. If pembrolizumab or another immunotherapy is also prescribed, use the matching combination regimen and its additional consent, monitoring and urgent advice. Pemetrexed with pembrolizumab maintenance is a different treatment plan.

### Treatment length

Treatment is planned every three weeks while it is helping and side effects remain acceptable. There is no fixed total number of cycles in this regimen. Scans, symptoms, blood results, your priorities and the oncology review guide continuation, delays, changes or stopping.

## Evidence depends on the treatment setting

Maintenance and second-line studies answer different questions.

Maintenance studies found longer disease control or survival with pemetrexed than placebo in selected patients whose cancer had not progressed after initial chemotherapy. One studied starting pemetrexed after a different chemotherapy combination; another studied continuing it after pemetrexed and cisplatin. A separate second-line study found similar survival to docetaxel with fewer important blood-count side effects. These historical comparisons do not predict an individual outcome or describe pemetrexed plus immunotherapy.

CHAPTER 02

# Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

## A three-week cycle within an ongoing treatment plan

**Pemetrexed Monotherapy**

21-day view




This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

DAY 1 INFUSION EVERY THREE WEEKS

### Pemetrexed alone: treatment, recovery and review

D1 D2 D3–D20 D21

Treatment is planned every three weeks while it is helping and side effects remain acceptable. There is no fixed total number of cycles in this regimen. Scans, symptoms, blood results, your priorities and the oncology review guide continuation, delays, changes or stopping.

BASELINE → TREATMENT → FOLLOW-UP

## Checks through your pemetrexed treatment

These columns show the order of care, not equal time intervals. Baseline tests and tests before every cycle are distinct. There is no fixed treatment length or scan interval; any additional immunotherapy needs its own monitoring plan.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00222 v5 · official regimen · EMA ALIMTA product information and patient leaflet

THE WHOLE TREATMENT JOURNEY

## Pemetrexed Monotherapy: treatment and checks

**Non-squamous non-small-cell lung cancer (NSCLC) · standalone maintenance or second-line chemotherapy**

One pemetrexed infusion on Day 1 of each 21-day cycle. Treatment is planned every three weeks while it is helping and side effects remain acceptable. There is no fixed total number of cycles in this regimen. Scans, symptoms, blood results, your priorities and the oncology review guide continuation, delays, changes or stopping.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting / phase change | Each 21-day cycle | Between visits | Benefit and toxicity review | After last pemetrexed |
|---|---|---|---|---|---|
| **Treatment course** NCCP 00222 | Confirm standalone maintenance or second-line | Day 1 pemetrexed; repeat every 21 days if appropriate | Recovery, daily folate and symptom reporting | Continue only while benefit and tolerability remain acceptable | Agree next treatment or follow-up plan |
| **Blood count, kidneys and liver** NCCP 00222 | Baseline full blood count (FBC), renal and liver profile | Repeat before each cycle | Earlier tests if clinically needed | Review recovery and any dose change | Follow unresolved abnormalities |
| **Vitamins and steroids** NCCP 00222 | Confirm folate, last B12 and one steroid schedule | Folate daily; B12 every three cycles; steroid around each dose | Report missed medicines or poor intake | Check adherence and changes to support medicines | Continue folate for 21 days after final dose |
| **Symptoms and medicine interactions** NCCP 00222 · SMPC | Review prior toxicity and all medicines | Assess interval symptoms before authorising treatment | Urgent contact for illness; check NSAID/aspirin use | Discuss prior reductions, nerve recurrence and daily function | Follow persistent kidney, nerve, blood or other problems |
| **Disease and treatment aim** NCCP 00222 | Confirm maintenance versus second-line indication | Review symptoms and clinical progress | Extra assessment if needed | Scans and tests according to Consultant plan | Agree follow-up or a different treatment plan |

## What each check is for

1

### Vitamin preparation or carry-forward

Before first pemetrexed or a phase change

Vitamins lower serious pemetrexed toxicity.

**Ask / act:** Confirm last B12 and next due date, daily folate and one steroid prescription.

2

### Review each dose

Before every 21-day cycle

Counts, organ function and toxicity can change.

**Ask / act:** Confirm whether to continue, delay, modify or stop.

3

### Benefit review

According to Consultant plan

There is no fixed total cycle number.

**Ask** / **act:** Confirm ongoing benefit and the next plan.

4

### Final dose follow-up

After the final pemetrexed

Protection and recovery continue after the infusion.

**Ask** / **act:** Agree medicine stop dates and review persistent symptoms.

**At the next review, ask**

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

## Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

### Before the visit

Confirm blood-test arrangements, transport, prescriptions, B12 booking and the 24-hour number. Tell the team if you are ill before travelling.

STEP 2

### Time in the unit

Pemetrexed is infused into a vein over about 10 minutes on Day 1. The whole visit is longer because it includes blood-result review, symptom checks, medicines, preparation and observation. The team can use a cannula or suitable existing line; this regimen has no routine take-home infusion pump.

STEP 3

### Home medicines

Keep one written plan for folic acid, B12, steroid and anti-sickness medicines. Ask the pharmacist to resolve duplicate or conflicting instructions.

STEP 4

### At a phase change

Confirm that the prescription is pemetrexed alone, review the anti-sickness plan, carry forward the appropriate vitamin schedule and ask whether other treatment needs separate information.

STEP 5

### Practical support

Arrange help with travel or daily tasks and bring a medicines list, symptom notes and questions. Extra passport pages can be provided while treatment continues.

## Before starting or changing to maintenance

A change in phase needs a clear new prescription and review.

### Checks before the dose

Blood count, kidney and liver tests are checked before starting and before each cycle. Tell the team about infections, breathlessness, bowel or mouth problems, rash, numbness and any previous severe treatment reaction.

### Confirm the vitamin schedule

Check how much folic acid you take and when your last B12 injection was given. If you already received pemetrexed, the team carries forward the correct vitamin schedule.

### Medicines review

Check with the team before using ibuprofen, other anti-inflammatory painkillers, aspirin or any new medicine. Kidney function and the medicine determine whether and when it needs interruption. Do not stop a prescribed medicine on your own; ask for a written plan.

### Your consent conversation

Discuss whether this is maintenance or second-line treatment, the individual aim, likely benefit and uncertainty, alternatives including not proceeding, common and serious harms, possible lasting kidney or nerve effects, fertility and pregnancy, vitamin and steroid medicines, practical burdens and your right to reconsider. A discussion aid supports but does not replace informed consent.

CHAPTER 03

# Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

## Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

| Treatment or check | Timing in this view |
|---|---|
| **Pemetrexed** |  **Day 1** Day 1 after clinical review |
| **Rash prevention** |  **Days 1–2** Steroid starts the day before treatment and continues through the day after |
| **Folic acid** |  **Days 1–21** Daily throughout treatment; B12 on its separate three-cycle schedule |
| **Report symptoms** |  **Days 1–21** Symptoms can occur at any time |
| **Next dose review** |  **Days 20–21** Blood tests and symptom review before the next authorised dose; appointments vary |

**Before the first pemetrexed:** Folate preparation and the first B12 injection start before the infusion. When already on pemetrexed, confirm the established vitamin schedule.

**Treatment length:** Treatment is planned every three weeks while it is helping and side effects remain acceptable. There is no fixed total number of cycles in this regimen. Scans, symptoms, blood results, your priorities and the oncology review guide continuation, delays, changes or stopping.

**Day before each dose:** Start the prescribed rash-prevention steroid the day before pemetrexed; the reference cycle starts at infusion Day 1.

**After the final dose:** Folate continues for 21 days after the last pemetrexed; persistent symptoms and follow-up remain important.

> **Why these bands stay broad:** This reference cycle shows care and checks, not predicted safe or high-risk days. Symptoms can occur at any time. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00222 v5 · official regimen ↗

**There are no guaranteed "safe days".** Follow your unit's emergency instructions and use its 24-hour number. Serious infection can occur without a high temperature; sudden illness or shivering still needs immediate advice.

**Between-treatment notes**

### Day 1: Infusion day

Pemetrexed is infused into a vein over about 10 minutes on Day 1. The whole visit is longer because it includes blood-result review, symptom checks, medicines, preparation and observation. The team can use a cannula or suitable existing line; this regimen has no routine take-home infusion pump. Report any reaction or discomfort around the drip immediately.

### Day 2: Day after pemetrexed

Continue the prescribed rash-prevention steroid through the day after treatment, folic acid daily, and anti-sickness medicine as directed. Use one reconciled medicine plan; do not double steroids.

### Day 3–20: Between visits

Continue folic acid and monitor symptoms. Your B12 injection is due every three cycles and may be booked on a treatment day. Call promptly for illness, poor intake, reduced urine, bowel or mouth symptoms; do not wait for review.

### Day 21: Review before another cycle

Blood count, kidney and liver tests, symptoms, previous toxicity and treatment benefit guide the next decision. Confirm the next dose, vitamins, support medicines and scan/review plan.

## Support around the medicines

**1** **Folic acid every day**

The source uses folic acid 350–1000 micrograms daily. Take at least five doses in the seven days before the first pemetrexed, continue throughout treatment and for 21 days after the final dose. Follow your own prescription and tell the team about missed doses.

**2** **Vitamin B12 injection**

The source uses vitamin B12 1000 micrograms into a muscle during the week before the first pemetrexed, then every three cycles. Later injections may be on treatment day. A change from platinum treatment to maintenance does not automatically restart the B12 clock: your team checks the last dose and the next due date.

**3** **One steroid prescription**

Pemetrexed rash prevention uses dexamethasone-equivalent 4 mg twice daily on the day before, day of and day after treatment. Your pharmacist or prescriber should give one clear steroid schedule that also accounts for any anti-sickness medicines. Do not add a second steroid course yourself.

### 4 Low-risk anti-sickness plan

Pemetrexed on its own has a low sickness risk. Take the prescribed prevention and rescue medicines and call if sickness persists. A previous platinum anti-sickness course should be reviewed when moving to pemetrexed alone.

### 5 Kidneys and other medicines

### 6 Reporting symptoms and getting help

Keep your 24-hour oncology number available, record symptoms and ask for practical or emotional support early.

### 7 Vaccines

Ask the oncology team before any vaccination. Some live vaccines are unsuitable during treatment; the team can advise on the right vaccine and timing.

## Treatment day

Assessment comes before dose authorisation.

### Your infusion

#### Tell the nurse immediately

Report feeling unwell, itching, swelling, wheeze, chest tightness or pain, burning or redness around the drip.

#### A delay or dose change

The team may delay or reduce a dose so that blood counts or symptoms can recover. A change does not itself mean the treatment has failed; ask why it is needed and when review is planned.

## Care between visits

Keep a short symptom record and ask for help early.

#### Vitamins and steroids

Follow the written folic acid, B12, steroid and anti-sickness plan. Tell the team promptly about missed doses or vomiting that prevents you keeping medicines down.

### Mouth, bowel and skin symptoms

Report mouth ulcers, difficulty eating or drinking, persistent diarrhoea, rash and changes in daily function. Severe symptoms or sudden illness need urgent advice.

### Daily life and support

Balance activity with rest. Ask for help with fatigue, eating, transport, work, finances or emotional wellbeing. Discuss what matters most to you at each review.

### Pregnancy, fertility and feeding

Tell the team immediately if pregnancy is possible. Discuss fertility preservation and contraception before treatment and confirm the medicine-specific duration after the final dose. Breastfeeding must stop during pemetrexed.

## When to get urgent help

Keep your oncology contact number available day and night.

### When you call

Give the medicine name, last treatment date and symptoms. Say whether you can drink, pass urine and manage everyday activities. Do not delay a call to collect these details.

### Symptoms after stopping

New or worsening symptoms after the last dose still need advice. Explain that you recently had chemotherapy.

## Review and the next plan

Treatment decisions consider benefit, harms and your priorities.



### Scans and disease review

Scans and other tests follow the Consultant treatment plan. There is no fixed scan interval in this regimen, and side effects do not reliably show whether treatment is working.

### After the final pemetrexed

Continue folic acid for 21 days after the last pemetrexed. Agree follow-up for persistent blood-count, kidney, nerve or other symptoms and confirm which support medicines stop.

## Vaccines and radiotherapy

Tell the team about planned vaccines and any past or planned radiotherapy.

**Check before a vaccine**

The team must review the vaccine type and timing. Yellow fever vaccine must not be given with pemetrexed; other live vaccines are also not usually suitable.

**Past or planned radiotherapy**

Tell the team, even if radiotherapy was years ago. Report new cough, breathlessness or a skin reaction in a previously treated area.

## Questions worth asking

**Does having more side effects mean treatment is working?**

No. Side effects do not reliably show whether the cancer is responding. Report them so they can be treated. Your team assesses benefit using the tests and review plan appropriate to your cancer.

**Why might a dose be delayed or reduced?**

Your team may allow recovery, investigate a symptom or reduce the chance of lasting harm. Ask which drug changes, why, and when review is planned. A delay does not itself mean treatment has failed.

**What should I say when calling?**

Give the regimen name, last treatment date, symptoms and when they started. Explain whether you can eat, drink, pass urine and manage usual activities. Give your temperature if available, but do not delay calling to collect details.

**What belongs in the consent conversation?**

The individual aim and likely benefit, uncertainty, reasonable alternatives including not proceeding, common and serious risks, lasting effects, fertility and practical concerns. You can ask for an interpreter, more time or another explanation and can reconsider your decision.

**How many cycles should I use this passport for?**

The passport provides reusable recording pages, not a prescribed course length. Your oncology team sets the duration and can provide extra pages.

**Can I use this page to decide if blood results are safe?**

No. Treatment decisions require the clinical protocol, symptoms, previous toxicity, organ function and the prescriber’s review. The patient page explains the checks and does not authorise treatment.

**What is maintenance treatment?**

Maintenance means continuing cancer treatment after the initial chemotherapy has finished and the cancer has not progressed. In this regimen pemetrexed is the only anti-cancer medicine. Your team confirms whether it is being continued from your earlier combination or started as a new maintenance medicine.

### Can I stop the vitamins when platinum treatment stops?

Do not stop them because platinum stops. Folic acid and B12 protect against pemetrexed toxicity and continue according to the pemetrexed plan. The team checks your existing vitamin schedule at a phase change.

### Can I have vaccinations while on treatment?

Check first with the oncology team. Live vaccines, including yellow fever vaccine, can be unsafe during treatment. Other vaccines may still be appropriate; do not assume every vaccine should be stopped.

> **Try explaining the plan back:** Can you explain whether this is maintenance or second-line treatment, which vitamin and steroid medicines continue, when B12 is due, and how to get urgent help?

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**Which treatment this guide covers** This guide covers pemetrexed on its own for maintenance or second-line treatment of selected non-squamous non-small-cell lung cancer. If pembrolizumab or another immunotherapy is also prescribed, use the matching combination regimen and its additional consent, monitoring and urgent advice. Pemetrexed with pembrolizumab maintenance is a different treatment plan.

**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Pemetrexed alone · treatment day

**Active medicines:** pemetrexed

**Emetogenic risk:** low

| Medicine | Dose / route | When |
|---|---|---|
| Ondansetron | 8 mg orally once daily | Treatment day (Day 1)<br>NCIS V8 low-risk reference. Review local policy, QT risk and individual suitability; antiemetic class guidance permits a 5-HT3 antagonist or corticosteroid. |
| Metoclopramide | 10 mg orally three times daily as required | Subsequent days, as prescribed<br>NCIS low-risk row; prescriber/pharmacy must specify appropriate duration and assess contraindications, renal adjustment and interacting medicines. |
| Folic acid | 350–1000 micrograms orally daily | At least five doses in seven days before first pemetrexed; throughout treatment and21 days after final dose<br>Continue across a platinum-to-pemetrexed phase change. |
| Vitamin B12 (hydroxocobalamin) | 1000 micrograms intramuscular (IM) | Week before first pemetrexed, then every three cycles<br>Later injections may be given on pemetrexed day; reconcile the existing schedule at phase changes. |

| Medicine | Dose / route | When |
|---|---|---|
| Pemetrexed rash prophylaxis: dexamethasone equivalent | 4 mg orally twice daily | Day before, day of and day after pemetrexed<br>One reconciled steroid prescription. Do not add the previous platinum antiemetic steroid course or duplicate this for antiemesis. |

## Additional phase-specific support

- Standalone pemetrexed is low emetogenic risk; review and replace the prior platinum support plan when that phase ends.
- Daily folate and B12 every three cycles continue while pemetrexed continues.
- Rash steroid is one reconciled prescription; no second antiemetic dexamethasone course is added.
- Review renal function and NSAID/aspirin interactions against the current SmPC.
- Routine granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) or antimicrobial prophylaxis is not mandated by NCCP00222; individualise after clinical risk or toxicity review.
- Pembrolizumab/pemetrexed maintenance requires the combination regimen and separate immune safety plan.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

# What to watch for

You do not need to know which medicine caused a symptom before calling.

### 1 Mouth

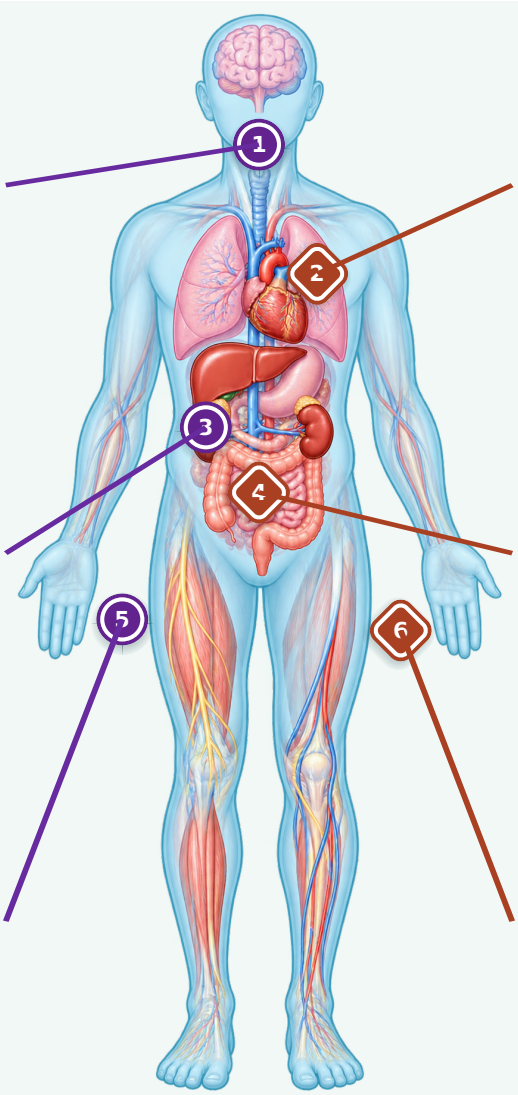
**Pemetrexed**

Report ulcers, painful swallowing or difficulty drinking.

### 2 Chest / breathing

**Pemetrexed**

New breathlessness, wheeze or chest pain needs urgent assessment; lung inflammation or infection can be serious.




### 3 Kidneys / fluids

**Pemetrexed**

Report reduced urine, dehydration or inability to drink; kidney tests guide treatment.

### 4 Stomach / bowel

**Pemetrexed**

Persistent vomiting or severe diarrhoea needs prompt advice; use prescribed anti-sickness medicines.

### 5 Hands / feet

**Pemetrexed / prior treatment**

Report new or worsening numbness, weakness, balance changes or difficulty with daily tasks.

### 6 Skin / blood counts

**Pemetrexed**

Report unusual bruising, bleeding or rash; fever or sudden illness needs immediate advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call your oncology 24-hour number immediately for a temperature of 37.5°C or above, 36°C or below, shivering or sudden illness. Call urgently for new breathlessness, chest pain, bleeding, severe or persistent diarrhoea, vomiting despite medicines, inability to drink, very little urine or a spreading or blistering rash. Do not wait for the next visit; serious infection can occur without a high temperature.

**When to get help**

**Call 112 / 999** for severe breathing difficulty, severe chest pain, collapse or a seizure.

**Call oncology immediately:** temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

CHAPTER 04

# Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

These recording pages are reusable and do not prescribe a fixed number of cycles. Ask for extra pages while treatment continues; your oncology team decides when the plan changes.

## Pemetrexed · Antifolate chemotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Mouth soreness / ulcers** | □ None | □ Mild soreness; eating normally | □ Painful; soft food needed: call | □ Cannot eat / drink enough: urgent review | □ Major breathing difficulty: 112/999 |
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/ day or at night: call | □ 7+ extra stools/ day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Skin / nail changes** | □ None | □ Mild change: report | □ Pain or daily tasks affected: call | □ Spreading infection or blistering: urgent review | □ Severe illness / collapse: emergency help |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

# Keep your treatment details to hand

**Non-squamous NSCLC · standalone maintenance or second-line chemotherapy** · NCCP 00222 v5 · Document revised 4 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

**FRONT / CONTACTS**

## My Treatment Card

SACTCheck

Name: _____

**Pemetrexed**

Day 1 every 21 days; duration reviewed

Phase: _____ Cycle plan: team to confirm

**Daytime (EXAMPLE)** 087 12345678

**Out of hours (EXAMPLE)** 087 87654321

**Unwell or fever? Call oncology now.**
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

**BACK / TREATMENT RECORD**

## My cycle record

Rolling record — follow your agreed treatment plan.

| Cycle | Date / initials |
|---|---|
| | |
| | |
| | |
| | |

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

CUT outer border

**FOLD centre — do not cut**

## Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

## Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

### iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

### Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

## Your local contacts and resources

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.32 · NCCP 00222 v5 · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

CHAPTER 05

# Sources and further information

## IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.


1. **National Cancer Control Programme (NCCP) 00222 v5 · official regimen**
   https://healthservice.hse.ie/documents/6935/222_V5_PEMEtrexed_Monotherapy_.pdf
2. **EMA ALIMTA product information and patient leaflet**
   https://www.ema.europa.eu/en/documents/product-information/alimta-epar-product-information_en.pdf
3. **HSE chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
4. **NCCP NCIS Medical Oncology antiemetics V8, Table 1 low-risk row, page 4**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
5. **Hanna 2004: second-line pemetrexed versus docetaxel**
   https://pubmed.ncbi.nlm.nih.gov/15117980/
6. **JMEN 2009: switch maintenance pemetrexed versus placebo**
   https://pubmed.ncbi.nlm.nih.gov/19767093/?dopt=Abstract
7. **PARAMOUNT final OS 2013: continuation maintenance after pemetrexed/cisplatin**
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